

MCPDP)

- An acceptance by the practitioners of broad personal responsibility for judgments made and acts performed within the scope of the autonomy implies i.e. a large measure of autonomy implies a correspondingly large measure of responsibility, the quality of which shows that professions are intellectual in character.

Nurses are responsible for all nursing services rendered to their clients.

- An emphasis on the service to be rendered rather than the economic gain to the practitioners, as the basis for the organization and performance of the social service delegated to the occupational group i.e. professional workers cannot avoid certain obligations regardless of their personal feelings.

Nursing is a calling and nurses do try as much as possible to prevent strike actions. If at all they are to go on strike, they make alternative plans available for their clients. Nurses provide their services to a substantial degree in the general public interest, receiving its compensation through limited fees.

- It has a clearly defined membership of a particular group with a view to safe-guarding the interests of the profession.

NANNM, ANA, ICN etc. are examples of group that safeguard the interest of nurses.

- It involves a code of ethics. This is required to enforce high standards of professional conduct.

Nursing code of ethics guide nurses in its relationships with clients, colleagues, and the public.

- It sets up its own professional organization.

Nursing and Midwifery Council controls the quality and quantity of nurses produced

- It assures its members a professional career.

This is achieved through the approved scheme of service of each country. E.g. NO II to NO I to SNO to PNO etc; and furthering education like graduate and post graduate degrees.

(Any 4 correct points discussed = 8 marks)

5b

i. ORGANIZING

Organizing may be defined as the arranging of component parts into functioning wholes. The purpose of organizing is to coordinate activities so



that a goal can be achieved. The terms “planning” and “organizing” are often used synonymously. In the management, planning is the determination of what is to be accomplished, and organizing is the determination of how it will be accomplished. However, most authors still describe the two processes with considerable overlap.

In the nursing process, planning includes writing objectives, setting priorities, and determining activities to meet the objectives. Thus, organizing may be considered part of the planning, even though it is not specifically identified. Planning, and thus organizing, may be viewed as being part of all processes, including the leadership process. Thus, planning and organizing may be said to answer the what, why, how, when, and where questions about specific activities.

There are six steps in the organizing process:

1. Establish overall objectives
2. Formulate supporting objectives, policies and plans
3. Identify and classify activities necessary to accomplish the objectives
4. Group the activities in light of the human and material resources available and the best way of using them under the circumstances
5. Delegate to the head of each group and the authorities necessary to perform the activities
6. Tie the groups together horizontally and vertically, through authority relationships, and information systems. (4 marks)

ii. DIRECTING

Directing is a function of the manager that gets work done through others. Directing includes five specific concepts; giving directions, supervising, leading, motivating, and communicating.

- Giving directions is the first activity and suggests that directions should be clear, concise and consistent and should confirm to the requirements of the situation. The manager should be aware of the tone of the directives. Different types of situations require different emphasis. For example, an emergency situation calls for different variation of voice than does a routine request.
- Supervising is concerned with the training and discipline of the work force. It also includes follow up to ensure the prompt execution of orders.
- Leading is the ability to inspire and to influence others to the attainment of objectives
- Motivating is the set of skills the manager uses to help the employee to identify his/her needs and finds ways within the organization to help satisfy them.
- Communicating: involves the what, how, by whom, and why of directives or effectively using the communication process. (4 marks)

iii. EVALUATION

Evaluation is the process of finding out the value of something • It is the process of determining the value or worth of objects of interest against standard of acceptability.

► Roles of Evaluation

1. It is a step ensuring the delivery of high quality health care.
2. It is an important tool for controlling health care expenditures and ensuring



accountability.

➤ Levels of Evaluation

- Effort- To assess the resources or capacities
- Performance- To assess the output or outcome of the program
 - Adequacy of performance- The extent to which performance meets the program objectives
- Effectiveness- The amount of the intended objectives that have been attained
 - Efficiency- The degree to which the program achieved its result at the lowest possible cost

➤ Steps of Evaluation

1. Decide what is to be evaluated.
2. Collect the information needed to provide the evidence.
3. Compare the results with the targets or objectives.
4. Judge to what extent the targets and objectives have been met.
5. Decide whether to continue the programme unchanged, to change it or to stop it.

➤ Types of Evaluation

- Process evaluation- measurements obtained during the implementation of program activities to control or assure or improve the quality of performance or delivery.
- Impact evaluation- focuses on the immediate observable effects of a program leading to the intended outcomes of a program
- Formative evaluation- measurements and judgments made on the process of the program.
- Summative evaluation (4 marks)

6a

i. HYSTERIA

Hysteria is a psychological disorder causing physical symptoms. It is a neurotic disorder characterized by violent emotional outbreaks and disturbances of sensory and motor functions. It is a shifting of emotional conflict, fear and anxiety into physical and mental symptoms.

Hysteria is a mental disorder which arises from intense anxiety. The patient loses control over his or her acts and emotions and it is usually accompanied by sudden seizures of unconsciousness with emotional outbursts. It is often due to repressed conflicts within the person. Though the disease may occur in both sexes, it is common in young women between fourteen to twenty five years of age. Hysteria is uncommon after the age of forty five years.

(2 marks)

ii. CLINICAL MANIFESTATIONS



A hysterical personality typically displays symptoms like seductive behavior, high level of emotional dependency, platonic friendships, intolerance, frustration, capriciousness and irritability. During fits, such a person becomes hyper-emotional. He exhibits exaggerated feelings like spells of crying and tantrums marked with symptoms like:

- Increasing Abdominal constriction
- Severe cramps and heaviness in the limbs
- Suffocation and headache
- Swelling of the neck
- Feeling of a foreign body lodged in the throat
- Laughing or crying without cause
- Gauser syndrome
- Palpitations
- Aphonia
- Clenched teeth
- Loss of appetite
- Mutism
- Hysterical trance

In severe cases the symptoms may be wild and painful cries, enormously swollen neck, incomplete loss of consciousness, violent movements, violent and tumultuous heartbeat and convulsions.

The hysteria patient usually has a weak will power, craving for love and sympathy and has a tendency towards emotional instability. Hysteria trances may last for days or weeks. A patient in trance may seem to be in deep sleep but the muscles are not usually relaxed. (Any 6 correct points listed = 3 marks)

iii. TYPES

- Conversion Hysteria
 - Dissociation Hysteria
 - Anxiety Hysteria
 - Hysterocatalepsy
- (Any 2 correct points listed = 1 mark)

iv. METHODS USED IN THE MANAGEMENT OF HYSTERIC PATIENTS

- Counselling
 - Family therapy
 - Behavioural therapy
 - Hypnosis
 - Abreaction
 - Psychoanalysis
 - Confrontation interview
 - Desensitization
 - Flooding
- (Any 4 correct points mentioned = 2 marks)

6b

i. TESTAMENTARY CAPACITY



Testamentary capacity is the specific ability of an individual to make a will. What is required to be established to certify that someone has testamentary capacity include:

- The individual must understand the nature of the making of a will, its effects and the extent of their property and possessions.
- The person must know the people who has claim on his bounty; he must be able to appreciate the claims to the estate, which he ought to consider.
- The person must have sufficient judgement and insight that he can determine the strength of the claims of his relatives and friends
- The person must also be “free of any disorder of the mind which would poison his affections, pervert his sense of right, or prevent the exercise of his natural faculties; that no insane delusion shall influence his will in disposing of his property and bring about a disposal of it which, if the mind had been sound, would not have been made”.

Since wills are usually contested after the testators death, forensic evaluations to determine Testamentary Capacity require careful review of medical, psychiatric, nursing home, pharmacy, and other relevant records in an attempt to reconstruct the mental state of the testator at the time of making the will. Friends, family members, acquaintances, and individuals who interacted with the testator around the time of making the will should be interviewed if available. (3 marks)

ii. POWER OF ATTORNEY

A power of attorney is a legal document which appoints one person (the attorney) to act on behalf of another (the donor) in areas of property and financial management. It is a written document in which one person (the principal) appoints another person to act as an agent on his or her behalf, thus conferring authority on the agent to perform certain acts or functions on behalf of the principal. The attorney is usually a family member or close friend. A power of attorney does not enable a person to make medical or lifestyle decisions on behalf of another. The person to make these decisions is an Enduring Guardian appointed in a separate document.

There are two types: ordinary power of attorney and enduring power of attorney. The difference is that an ordinary power of attorney ceases to have effect when a person loses mental capacity but an enduring power of attorney continues in force after that time.

Anyone can develop conditions like dementia or Alzheimer or suffer a stroke or be injured in a car accident so that they are unable to manage their financial affairs and property. Making an enduring power of attorney is a practical step that one can take while one still have legal and mental capacity. It means that essential financial matters can be looked after by someone on one's behalf if one loses capacity.



Durable powers of attorney have become popular because they enable the principal to have his or her affairs easily and inexpensively after he or she becomes incapacitated. With durable powers of attorney, a principal can appoint someone to handle his or her affairs after he or she becomes incompetent. (3 marks)

iii. SYSTEMATIC DESENSITIZATION

Systematic desensitization, also known as graduated exposure therapy, is a behavioral technique commonly used to treat fear, anxiety disorders and phobias. Using this method, the person is engaged in some type of relaxation exercise and gradually exposed to an anxiety producing stimulus, like an object or place.

It is a behavioral technique whereby a person is gradually exposed to an anxiety-producing object, event, or place while being engaged in some type of relaxation at the same time in order to reduce the symptoms of anxiety.

There are three main steps to successfully desensitize an individual:

- Establish anxiety stimulus hierarchy: The individual must first identify the items that are causing anxiety.
- Relaxation Training: First, a clinician or behavioral therapist would train the client with the fear of flying in relaxation techniques.
- Hierarchy of Fears: The next step involves constructing what is called a hierarchy of fears, which is a list of the things that the person identifies as fearful related to flying in order from the least to the most anxiety-provoking. (3 marks)

iv. SENILE DEMENTIA

Senile dementia is a progressive, irreversible, degenerative neurological disorder that begins insidiously. It is progressive loss of cognitive function, marked by memory problems and confused thinking. The most common form of dementia is Alzheimer's disease. Dementia most often occurs during old age, but is a more severe form of decline than that of normal aging.

Dementia is a loss of mental ability severe enough to interfere with normal activities of daily living, lasting more than six months, not present at birth, and not associated with a loss or alteration of consciousness. It is a group of symptoms caused by gradual death of brain cells. The loss of cognitive abilities that occur with dementia leads to impairments in memory, reasoning, planning and behavior.

Alzheimer's disease is the most common cause, followed by vascular or multi-infarct dementia. Other conditions that may cause dementia include AIDS, Parkinson's disease, Lewy body disease, Pick's disease, Huntington's disease, Creutzfeldt-Jakob disease, brain tumor, hydrocephalus, head trauma, multiple sclerosis, prolonged abuse of alcohol or other drugs,



vitamin deficiency: thiamin, niacin, or B12, hypothyroidism and hypercalcemia. Symptoms of dementia include memory loss, Impaired abstraction and planning, language and comprehension disturbances, poor judgment, impaired orientation ability, decreased attention and increased restlessness. This may cause the person with dementia to begin an activity and quickly lose interest and/or to wander frequently, behavioral changes and psychosis.

Nursing intervention is aimed at maintaining physical safety, reducing anxiety and agitation, improving communication, promoting independence in self-care activities, ensuring adequate nutrition, managing sleep pattern disturbance and educating family care givers. (3 marks)

